

## **Unconsidered Metrics**

There are many other ways that healthcare workers can check the vital signs of a patient besides the ones that we have already covered in the previous session, one of those being the Capillary Refill Time (CRT). The CRT is a quick, non-invasive physical test done by pressing down on the patient's nail bed or skin until it turns white, then releasing it. This is used to measure the time it takes for the blood to return to the tiny blood vessels (capillaries) at the surface of the skin after pressure is removed.

The normal capillary refill time is less than 2 seconds for a healthy individual with good circulation, which means that the heart is pumping blood successfully to the farthest extremities (fingers or toes). However, if there is a delayed response of more than 2 seconds, it is determined that the blood flow is sluggish. In that case, health care workers can deduce that either the patient is severely dehydrated or was exposed to extreme cold (maybe a case of hypothermia) or their blood vessels are narrowing due to a medical emergency.

Therefore, in a triage setting, CRT is an excellent tool for spotting early cardiovascular collapse. When a patient is entering the early stages of shock—whether from severe bleeding, a major infection, or dehydration—the body tries to protect itself by pulling blood away from the skin and redirecting it to vital internal organs like the brain and heart. However, with this test, certain complications may occur due to the fact that this test may be biased towards persons of colour. This is due to the fact that the test heavily relies on visually detecting a change from “blanched” back to “pink” and high levels of melanin can mask these subtle colour shifts. To overcome these limitations, healthcare workers can check the fingernail beds, the palms and soles, or even the inside of the lip of a patient who is of colour.